

Nosebleed - Female (Child)

Algorithm 1028 | Child (0-17 years) | Gender at birth: Female

Definition

- Nosebleed assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, are they pregnant, anticoagulated, or known to have a bleeding disorder, how long/severely has bleeding continued despite uninterrupted pressure, and was there trauma, surgery, foreign body, or safeguarding concern?
2. [YES_NO] Did the bleeding start after a blow to the head?
3. [YES_NO] Is the caller taking blood-thinning medication such as warfarin?
4. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Nose Injury

Did bleeding begin after a significant facial or nasal injury?

A traumatic nosebleed requires the Nose Injury pathway.

1. Disposition level 100

Is there difficulty breathing, reduced consciousness, collapse, faintness or shock, major facial/head trauma, or severe uncontrolled bleeding that is rapidly worsening?

Immediate life-threatening features require Qatar 999. This separates ambulance need from other NHS.UK an emergency assessment service selected through the Qatar pathway approved by governance (GOVERNANCE_REQUIRED) criteria that may be stable enough for governed non-ambulance transport.

1. Disposition level 70

After uninterrupted pinching of the soft nose, has bleeding continued longer than 10-15 minutes, does it still seem excessive, is the person swallowing a lot of blood and vomiting, or did it begin after a blow to the head without the immediate life-threatening features above?

NHS.UK directs these features to an emergency assessment service selected through the Qatar pathway approved by governance (GOVERNANCE_REQUIRED) and says not to self-drive, while allowing either another driver or an ambulance. Exact Qatar destination and transport choice remain GOVERNANCE_REQUIRED.

1. Disposition level 50

Is the patient a child under 2, does the caller have regular nosebleeds, anaemia symptoms, take blood-thinning medication, or have a clotting disorder such as haemophilia?

NHS.UK guidance recommends a clinical review through a Qatar pathway approved by governance (GOVERNANCE_REQUIRED) for these situations even when the bleed itself has stopped.

1. Disposition level 15

Is this a single, brief nosebleed with none of the features above?

NHS.UK guidance describes standard first aid as sufficient for an uncomplicated nosebleed.

Advice

Qatar emergency nosebleed response

- Call Qatar emergency services on 999 and do not drive for severe/uncontrolled bleeding, breathing compromise, faintness/shock, major facial/head trauma, or reduced consciousness. Sit forward if alert, spit blood out, and pinch the soft nose continuously. Do not tilt the head back, lie flat, pack the nose deeply, or repeatedly release pressure to check. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: bleeding remains uncontrolled; difficulty breathing, swallowing large amounts of blood, faintness, or shock; major trauma or reduced consciousness.

Urgent in-person nosebleed assessment

- Continue sitting forward and pinching the soft nose while arranging prompt in-person emergency assessment through the Qatar governance-approved pathway. Do not self-drive. A safe adult may transport a stable patient only if the approved local pathway permits; call 999 for breathing difficulty, reduced consciousness, collapse, shock, major trauma, or rapidly worsening/uncontrolled bleeding. Exact destination and transport remain GOVERNANCE_REQUIRED.
- Call Back If: difficulty breathing, faintness, collapse, or reduced consciousness; bleeding becomes rapidly worse or remains uncontrolled; major trauma.

Routine nosebleed follow-up

- Arrange primary-care review through the approved Qatar pathway for recurrent nosebleeds or bleeding-risk factors.

- Call Back If: nosebleeds becoming more frequent or heavier.

Home first aid for a nosebleed

- Sit down, lean forward with the head tilted forward, pinch the soft part of the nose just above the nostrils for 10-15 minutes, and breathe through the mouth. An ice pack may help.
- Call Back If: bleeding continues past 10-15 minutes of pinching; feeling weak or dizzy.

First aid

- Call Qatar emergency services on 999 and do not drive for severe/uncontrolled bleeding, breathing compromise, faintness/shock, major facial/head trauma, or reduced consciousness. Sit forward if alert, spit blood out, and pinch the soft nose continuously. Do not tilt the head back, lie flat, pack the nose deeply, or repeatedly release pressure to check. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Nosebleed", <https://www.nhs.uk/conditions/nosebleed/> (page last reviewed 05 December 2023)

Provenance

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